A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

29 April 2026

A/Prof Pav Nanayakkara
Jean Hailes For Womens Health

Dear A/Prof Nanayakkara,

Re: Miss Chanelle Edwards
DOB: 08/05/2007 Phone: 0423 860 198
Address: 7 Kingfisher Drive West, MOAMA, NSW, 2731
Email: chanelle.r.edwards@icloud.com

Summary

- **Chronic abdominal pain, nausea, and altered bowel habit.**
- **Arranged for gastroscopy, colonoscopy, and autoimmune blood screening.**
- **No action required by you.**

I had the pleasure of conducting an Telehealth consultation with Chanelle thank you for referring her.

Symptoms and history of presenting complaint

Chanelle is an 18-year-old woman with chronic gastrointestinal symptoms. Since late December, she has been experiencing significant abdominal pain associated with her bowel movements. She describes an alternating bowel habit of constipation and diarrhoea, often accompanied by sudden faecal urgency. She also reports persistent bloating and nausea, noting that the abdominal pain is occasionally exacerbated by coughing. Interestingly, she recalls a similar episode of pain approximately three years ago, at which time a CT scan reportedly identified mesenteric adenitis. She denies any history of haematochezia or mucus in her stool.

Diet

Chanelle's diet is somewhat irregular; she frequently skips breakfast and consumes tea throughout the morning. She denies alcohol consumption but does occasionally consume snacks and confectionery. There are no known specific food intolerances identified at this stage.

Past medical history and medications

Chanelle's medical history is significant for a previous diagnosis of mesenteric adenitis. She denies major psychiatric disorders. She does not take any regular prescription medications, though she uses ondansetron as required for nausea. She also reports the infrequent use of non-steroidal anti-inflammatory drugs (NSAIDs) for pain relief.

Social history, family history

Chanelle is currently 18 years old works full-time as a hairdresser she lives with her partner and her family.. Her maternal grandmother has a known diagnosis of coeliac disease. There is no known family history of inflammatory bowel disease, colorectal cancer, or other gastrointestinal malignancies.

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Examination

I couldn't perform an examination as this was a telehealth appointment.

Impression and plan

Chanelle presents with a complex history of abdominal pain and bowel dysfunction. While her symptoms may be functional in nature—possibly relating to visceral hypersensitivity or anterior cutaneous nerve entrapment syndrome (ACNES) given the pain on coughing—organic pathology must be excluded. This is particularly relevant given her family history of coeliac disease and the chronicity of her symptoms.

- **Investigations:** I have arranged for a gastroscopy and colonoscopy to be performed at a day centre on the 21st of May. We will obtain biopsies to screen for coeliac disease, *Helicobacter pylori*, and microscopic colitis.
- **Pathology:** I have requested a comprehensive suite of blood tests, including an autoimmune screen, to look for underlying inflammatory or systemic conditions.
- **Management:** If the investigations are unremarkable, I have discussed the potential for a functional gut-brain axis disorder. In that case, we may trial a course of low-dose neuromodulators to desensitise the enteric nerves, typically for a duration of nine to twelve months.

Thank you again for referring this patient.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

All appointments and enquires

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Endoscopy locations

Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065